LEISHMANIASIS:

Epidemiological report on the Region of the Americas

No. 13, December 2024

Pan American Health Organization
World Health Organization
Americas Region

PAHO

Introduction

The *Plan of action to strengthen surveillance and control of leishmaniasis in the Americas 2023–2030 (1)* was published in 2024 in order to advance toward the goals established in the *PAHO Disease Elimination Initiative: A policy for an integrated sustainable approach to communicable diseases in the Americas (2)* of the Pan American Health Organization (PAHO) and the *Roadmap for neglected tropical diseases 2021–2030 (3)* of the World Health Organization (WHO), as well as to update, systematize, and monitor actions related to leishmaniasis at the most disaggregated operational level.

The proposed actions were formulated on the basis of a diagnosis of the situation in the Region of the Americas, which involved defining goals, process, and epidemiological indicators, as well as establishing baselines for assessing progress and monitoring the disease with a view to achieving concrete improvements.

PAHO's *Guideline for the treatment of leishmaniasis in the Americas*, Second edition (2022) *(4)* recommends the use of local treatments for uncomplicated cutaneous leishmaniasis (CL). The efficacy of local treatments is similar to that of systemic antimonials. Local treatments also have a better safety profile, reduce treatment time, and can be implemented in primary health care settings and in remote and hard-to-reach areas.

Starting in 2023, in collaboration with the United States Agency for International Development (USAID) and the Drugs for Neglected Diseases Initiative (DNDi), PAHO launched the implementation of local treatment of leishmaniasis in the endemic countries of the Region. Thermotherapy devices were donated and health professionals from endemic countries received training.

Figure 1 shows the countries that, as of October 2024, had included thermotherapy as a treatment alternative for uncomplicated CL in endemic areas, and for use in remote and hard-to-reach areas.

Figure 1. Countries that have included thermotherapy as a treatment alternative for uncomplicated cutaneous leishmaniasis in endemic, remote, and hard-to-reach areas, Region of the Americas, 2024

To access an infographic with 2023 data on cutaneous and visceral leishmaniasis in endemic countries in the Region of the Americas, click on the countries in Figure 1.

CL: cutaneous leishmaniasis
Source: PAHO.

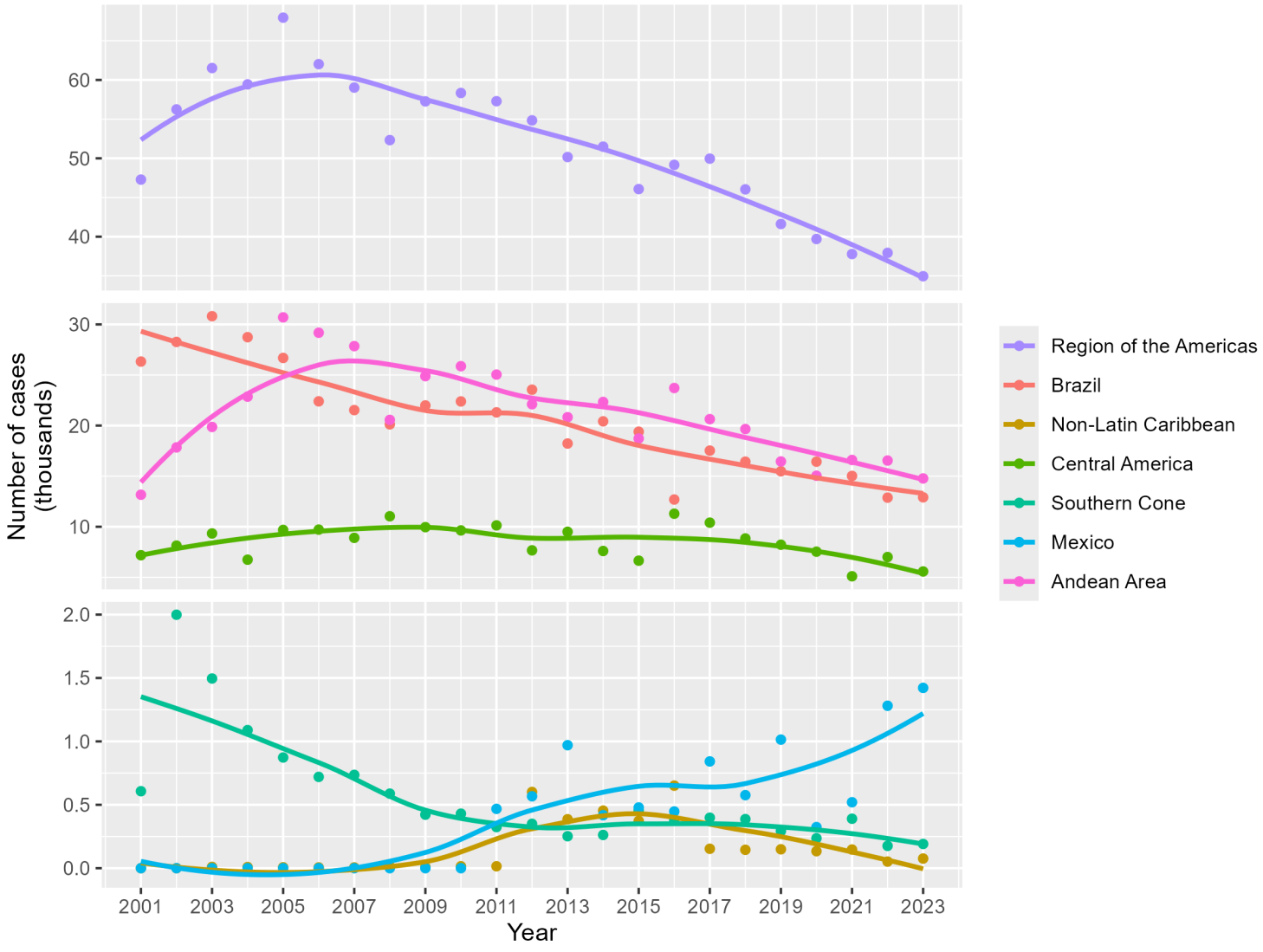
Epidemiological scenario

Cutaneous and mucosal leishmaniasis

During the 2001–2023 period, 17 countries in the Region reported to PAHO 1 105 545 cases of cutaneous leishmaniasis (CL) and mucosal leishmaniasis (ML), with an average of 52 645 cases per year (Table 1).

This disease has been on a downward trend since 2005, with 34 954 cases reported in 2023, the lowest number observed in the period (Figure 2). A comparison of the data for 2023 with the previous year shows an 8% overall decline in the number of reported cases. This decrease was observed in all countries of the Region except Argentina (15%), Costa Rica (36%), Ecuador (18%), Guyana (100%), Mexico (11%), and Suriname (40%), where the number of cases increased in 2023.

Figure 2. Number of reported cutaneous leishmaniasis cases, Region of the Americas, 2001–2023



Note: Data reported by countries' leishmaniasis programs and surveillance services for 17 endemic countries in the Region of the Americas. The graphed points correspond to counts of actual cases. The lines correspond to short-term trends, calculated by local regression.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

In 2023, the incidence rate in the Region was 15.4 per 100 000 population. The countries with the highest incidence (per 100 000 population) were Suriname (80.8 cases), Guatemala (43.8), Peru (37.8), Panama (36.9), the Plurinational State of Bolivia (35.6), and Nicaragua (32.8). The countries with the lowest rates per 100 000 population were Argentina (2.7 cases), Ecuador (8), Paraguay (4.5), and the Bolivarian Republic of Venezuela (8.8).

Table 1. Historical series of cases of cutaneous, mucosal, and visceral leishmaniasis, Region of the Americas, 2001–2023

Cutaneous leishmaniasis

	2001	2002	2003	2004	2005	2006	2007	2008	2009	2010	2011	2012	2013	2014	2015	2016	2017	2018	2019	2020	2021	2022	2023
Americas	47 286	56 243	61 518	59 439	67 949	62 017	59 027	52 324	57 264	58 337	57 287	54 842	50 163	51 491	46 074	49 165	49 959	46 041	41 617	39 705	37 786	37 937	34 954
Argentina	157	748	348	358	282	257	201	208	163	166	140	173	90	138	334	241	306	303	241	182	337	117	135
Bolivia (Plurinational State of)	2043	2518	2452	2819	2657	3152	3153	1838	1218	1809	1636	1767	2016	1683	2231	2222	2283	3127	2052	2059	2166	2197	2097
Brazil	26 328	28 268	30 812	28 737	26 685	22 397	21 530	20 123	21 989	22 387	21 306	23 547	18 226	20 418	19 395	12 690	17 526	16 432	15 484	16 432	15 023	12 878	12 910
Colombia	4130	7038	9267	10 698	18 043	16 241	13 331	9595	15 420	14 818	9684	9757	9353	11 586	7541	10 966	7764	6362	5907	6161	6175	5685	4898
Costa Rica	425	690	948	1061	1676	1870	1807	818	2025	1143	1376	1453	1950	2150	1171	1148	2224	1247	601	528	563	492	668
Ecuador	1754	1253	1336	2494	1925	1536	1185	1479	1735	1629	965	1512	873	1175	1479	1197	1632	1237	1104	1047	1251	875	1036
El Salvador	18	46	24	76	24	46	36	31	…	4	17	21	16	29	20	13	44	50	230	39	50	39	23
Guatemala	…	1549	1143	870	1243	602	287	494	519	410	549	572	664	258	562	835	775	1044	1167	1121	836	1134	869
Guyana	…	…	10	9	7	6	6	14	9	15	15	7	4	64	132	396	21	27	19	12	3	5	10
Honduras	957	1260	1684	797	1574	1300	855	1759	1502	1362	1736	1927	2074	1936	2040	2671	1854	1636	1985	1467	1119	1580	1194
Mexico	…	…	…	…	…	…	…	…	…	…	468	567	970	418	479	447	842	576	1014	324	520	1281	1422
Nicaragua	2924	2200	3716	2103	3521	2125	3719	5826	4047	3497	3235	1884	3035	1649	1925	5423	4343	3722	3321	3443	1251	2423	1532
Panama	2862	2390	1821	1837	1649	3774	2199	2109	1866	3221	3221	1811	1762	1581	930	1198	1164	1143	920	938	1286	1339	1294
Paraguay	450	1251	1148	731	591	463	535	380	259	264	184	177	162	124	122	135	92	84	52	54	53	59	56
Peru	5238	7032	6809	6849	8072	8248	10 183	7650	6512	7612	11 204	6969	6948	6231	5459	7271	6631	6321	5349	4178	5201	5756	5204
Suriname	…	…	…	…	…	…	…	…	…	…	…	594	382	390	241	255	132	118	130	122	144	47	66
Venezuela (Bolivarian Republic of)	…	…	…	…	…	…	…	…	…	…	1551	2104	1638	1661	2013	2057	2326	2612	2041	1598	1808	2030	1540

Visceral leishmaniasis

	2001	2002	2003	2004	2005	2006	2007	2008	2009	2010	2011	2012	2013	2014	2015	2016	2017	2018	2019	2020	2021	2022	2023
Americas	2552	2460	3118	3707	3694	3781	3759	3971	3858	3933	4059	2892	3395	3624	3390	3281	4228	3562	2603	1988	1799	1760	1604
Argentina	0	0	0	0	0	2	17	19	18	21	15	24	6	11	8	11	9	2	9	11	16	11	28
Bolivia (Plurinational State of)																		0	1	2	2	22	4
Brazil	2549	2450	2971	3580	3597	3651	3604	3852	3693	3716	3894	2770	3253	3453	3223	3127	4103	3466	2529	1933	1683	1684	1461
Colombia	0	0	121	96	66	44	54	33	54	34	11	9	13	31	21	37	29	16	11	8	7	10	5
Costa Rica	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
El Salvador	2	0	4	4	0	0	1	0	0	0	1	0	1	0	0	0	2	3	0	0	1	0	0
Guatemala	0	0	0	0	0	0	0	1	1	2	2	0	1	0	2	2	2	4	1	0	1	1	0
Honduras	0	0	0	0	7	9	4	3	3	7	7	0	3	2	6	7	8	8	3	0	0	0	1
Mexico	0	6	13	3	3	9	9	9	7	9	0	4	4	0	1	0	1	0	1	0	0	0	3
Nicaragua	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
Paraguay	1	4	9	24	21	66	70	54	82	144	114	76	107	118	92	64	34	19	22	26	58	72	95
Uruguay																		1	3	2	1	2	0
Venezuela (Bolivarian Republic of)	0	0	0	0	0	0	0	0	0	0	15	9	7	9	37	33	40	43	23	6	30	32	7

…: no data

Note: Data reported by countries' leishmaniasis programs and surveillance services.

Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

No significant difference was observed between the number of health units at the first and second subnational administrative levels that reported cases in 2023, compared to the previous year (in 2023, cases were reported in 233 departments, states, cantons, etc.; and cases were reported in 2902 municipalities, cantons, provinces, districts, etc.). Neither were there major differences in the proportion of cases at international borders (21%). Figures 3 and 4 present a regional analysis of CL and ML data for 2023, including cases and incidence, disaggregated to the second subnational administrative level.

Figure 5 shows a map with risk stratification based on the three-year composite indicator. A 37% decline was observed in municipalities with very intense, intense, and high transmission, reflected by a 53% decline in Brazil. Meanwhile, an increase was observed in municipalities with the highest transmission levels in the Plurinational State of Bolivia (63%), Colombia (15%), and Peru (73%), suggesting the need to reevaluate and plan surveillance and control actions according to the transmission level.

Figure 3. Cutaneous and mucosal leishmaniasis cases, second subnational administrative level, Region of the Americas, 2023

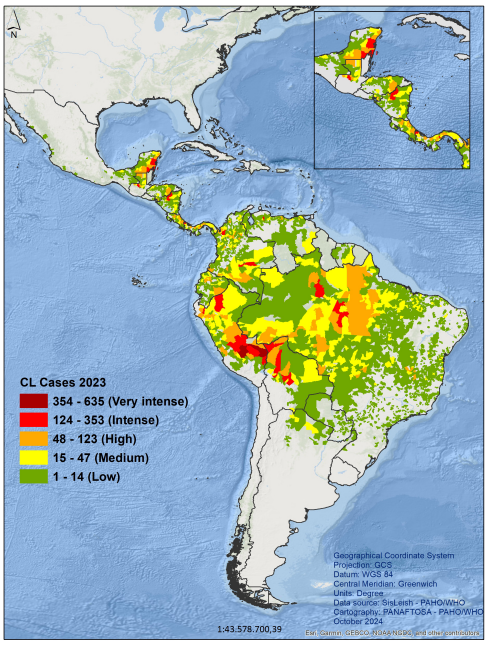


Figure 4. Incidence of cutaneous leishmaniasis per 100 000 population, second subnational administrative level, Region of the Americas, 2023

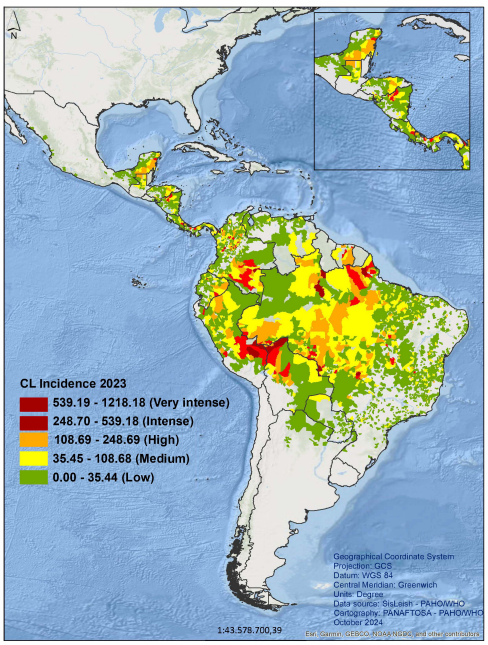
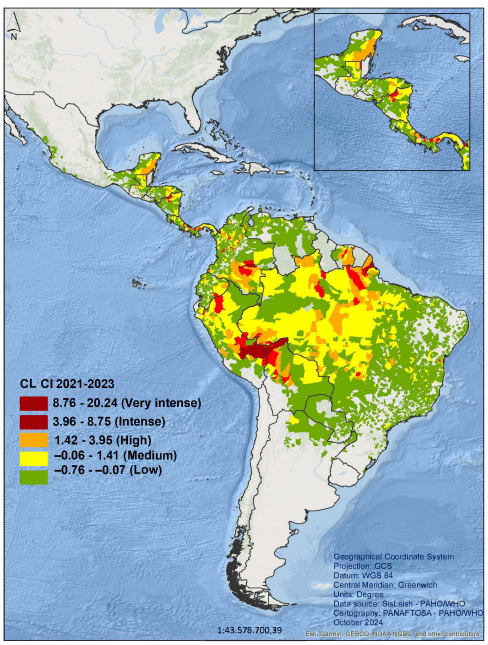


Figure 5. Cutaneous leishmaniasis composite indicator, second subnational administrative level, stratified by transmission risk, Region of the Americas, 2021–2023



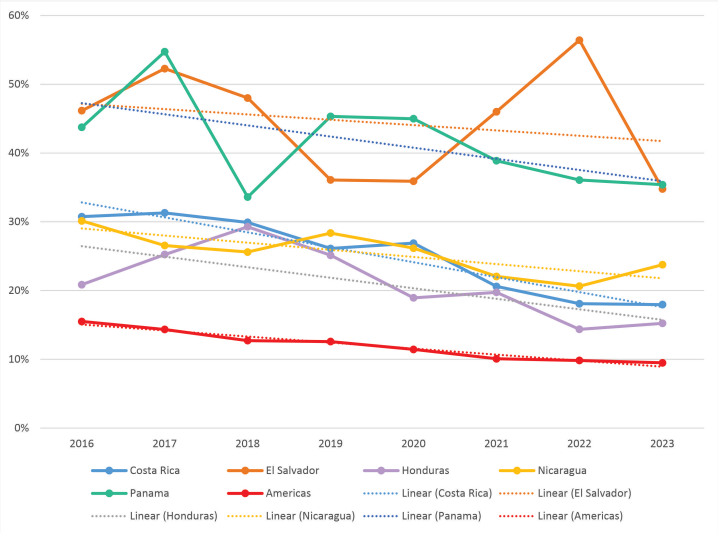
CL: cutaneous leishmaniasis.
CLCI: cutaneous leishmaniasis composite indicator, represented by average number of cases and incidence of cases per 100 000 population in the 2021–2023 triennium.
Note: Data reported by countries' leishmaniasis programs and surveillance services. Guyana is not represented in the figure because its political-administrative organization includes only a single subnational administrative level (regions).
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Of the total cases reported to SisLeish *(5)*, age and sex variables are available for 99.7% (34 841) and 99.9% (34 942) of reported cases, respectively. A sylvatic transmission pattern continues, with 51% of cases in the 20–50 years age group, and a 3:1 ratio between men and women with the disease.

The age group of children under 10 years is an important indicator for epidemiological and entomological surveillance of CL, since it reflects household or peridomiciliary transmission. This type of transmission can be addressed with different surveillance and control actions, which is why one of the targets of the *Plan of action to strengthen the surveillance and control of leishmaniasis in the Americas 2023–2030 (3)* is to reduce the proportion of new cases of CL detected and reported in this age group by at least 50%. In 2023, 9% of the cases reported in the Region were in this age group, the lowest figure since 2012.

In addition, there was a 39% reduction in the proportion of CL cases in children under 10 years of age compared to 2016, the year with the highest proportion of cases recorded in this period (16%), reflected by a reduction in all endemic countries except Guatemala, Paraguay, and the Bolivarian Republic of Venezuela, which saw an increase. Central American countries, with the exception of Guatemala, continue to have the highest proportions in the Region (more than 20% of cases), although a downward trend is observed throughout the period analyzed (Figure 6).

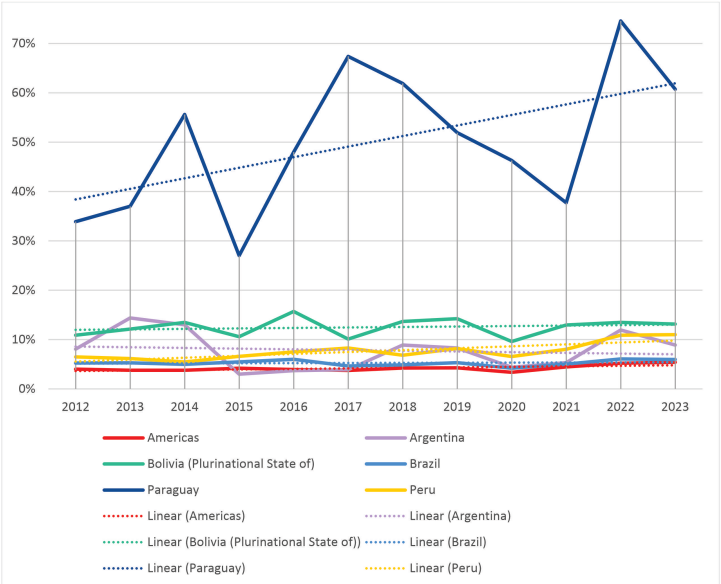
Figure 6. Countries with the highest proportion of cutaneous and mucosal leishmaniasis cases in children under 10 years old, Region of the Americas, 2016–2023



Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Regarding the clinical form, 97% (33 903) of reported cases included this variable in SisLeish. Of the total number of cases, 91.6% (32 016) corresponded to the cutaneous form and 5.4% (1 887) to the mucosal (ML) or mucocutaneous (MCL) form, the highest number reported since 2012, with an upward trend in the Region. In the countries that historically have had a higher proportion of cases of ML/MCL, a significant upward trend in this clinical form is observed in Paraguay, the country with the highest proportion, followed by the Plurinational State of Bolivia and Peru, with a slight upward trend; in Argentina and Brazil, a downward trend can be seen (Figure 7).

Figure 7. Countries with the highest proportion of mucosal leishmaniasis cases, Region of the Americas, 2012–2023

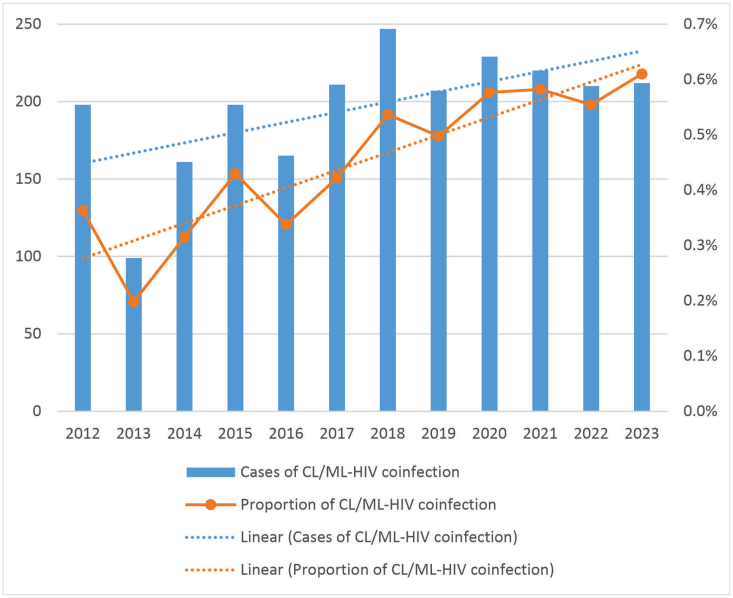


Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

As for other clinical forms, a total of 32 new cases of atypical cutaneous leishmaniasis (ACL) were reported in 2023: 21 in El Salvador and 11 in Nicaragua. No new cases of diffuse cutaneous leishmaniasis were reported in SisLeish. Information on clinical type was not available for Costa Rica or for 29% of cases in Panama.

Regarding the proportion of CL and human immunodeficiency virus (HIV) coinfection, a total of 212 cases were reported in 2023 in Argentina (1 case), Brazil (169), Colombia (35), Mexico (3), Paraguay (3), and the Bolivarian Republic of Venezuela (1). The proportion of reported cases was low over the years, but there is an upward trend in both the number of cases and the proportion of cases of CL/HIV coinfection (Figure 8).

Figure 8. Proportion of visceral leishmaniasis and human immunodeficiency virus (HIV) coinfection and number of visceral leishmaniasis cases in the Region of the Americas, 2012–2023

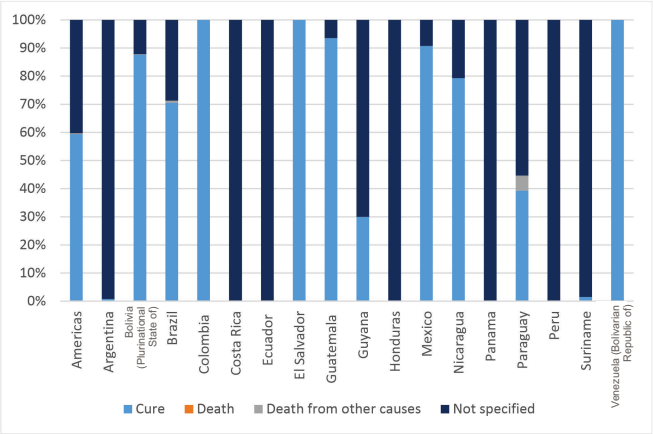


CL/ML-HIV: cutaneous/mucosal leishmaniasis and human immunodeficiency virus coinfection.
Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Of the total cases of CL and ML, 86% (29 966) were laboratory-diagnosed and 12% (4232) were diagnosed by clinical criteria and epidemiological link; in 2% of cases (756), this information was not available. The latter figure, the lowest in the Region since 2017, represents an 80% improvement in the completeness of this variable compared to 2019 (10%), reflecting improved data reporting in Guatemala, Honduras, Panama, and Suriname. Costa Rica continues to underreport this variable.

In the Region of the Americas, in 40% of cases (14 098), information on clinical outcome is not specified. In Argentina, Costa Rica, Ecuador, Honduras, Panama, Peru, and Suriname, clinical outcome is not available for all cases. Of the cases for which this variable was reported, 67% (20 763) progressed to cure, and 12 CL-related deaths were reported (75% in persons over 50 years of age) (Figure 9). These deaths need to be investigated by the countries' surveillance service.

Figure 9. Proportion of cutaneous and mucosal leishmaniasis cases by clinical outcome, Region of the Americas, 2023



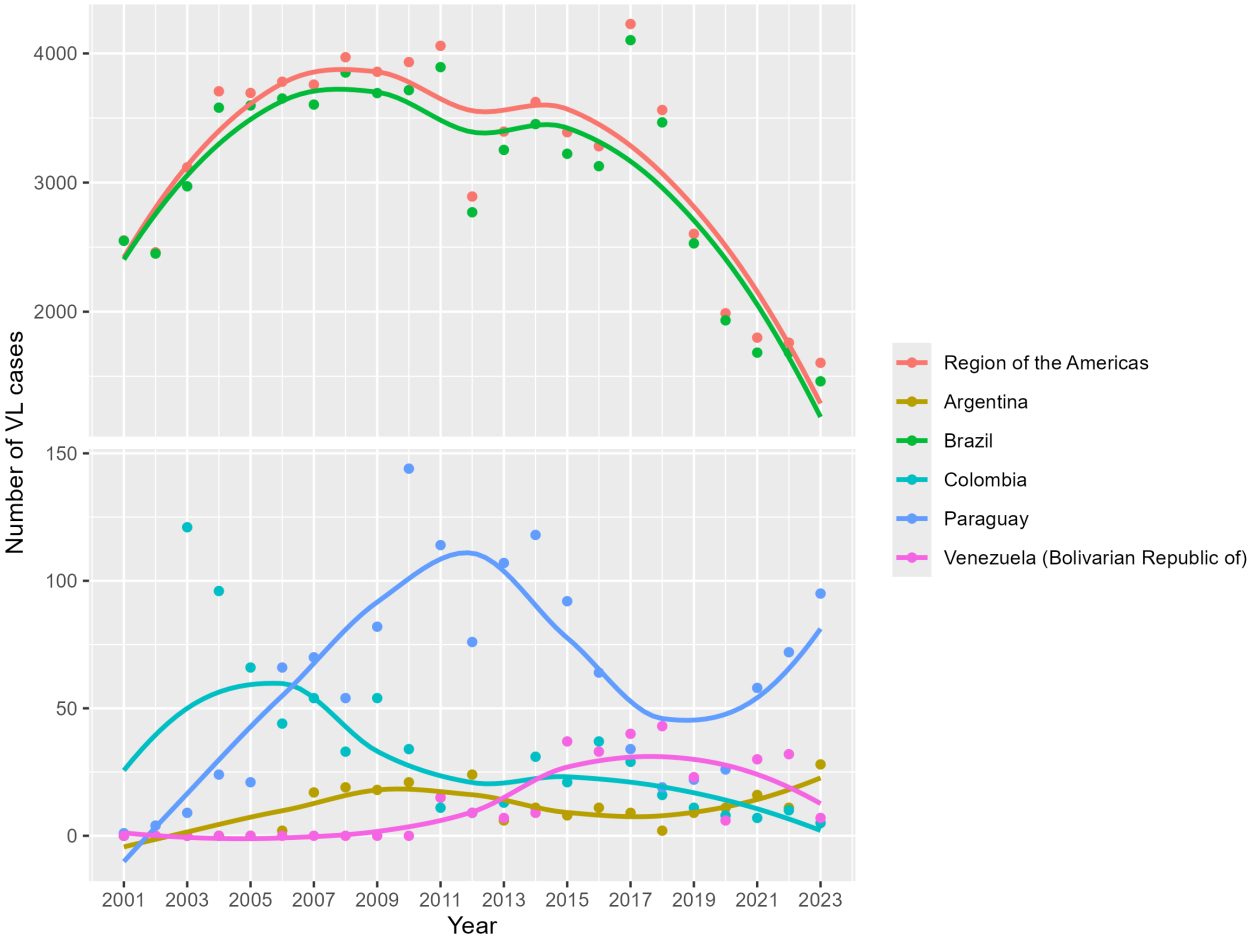
Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Visceral leishmaniasis

In the 2001–2023 period, a total of 73 092 new cases of visceral leishmaniasis (VL) were recorded in the Region of the Americas, with an average of 3178 cases per year (Table 1).

Since 2017, a downward trend in cases has been observed, with a 164% reduction in relation to the 1604 cases reported in 2023, the lowest number of recorded cases of VL in the historical series (Figure 10). Among the countries with continuous transmission over the years, a significant decline in cases was observed in Brazil, Colombia, and the Bolivarian Republic of Venezuela during this period. In Argentina and Paraguay there was an upward trend in 2023, with increases of 65% and 68%, respectively.

Figure 10. Number of reported visceral leishmaniasis cases, Region of the Americas, 2001–2023

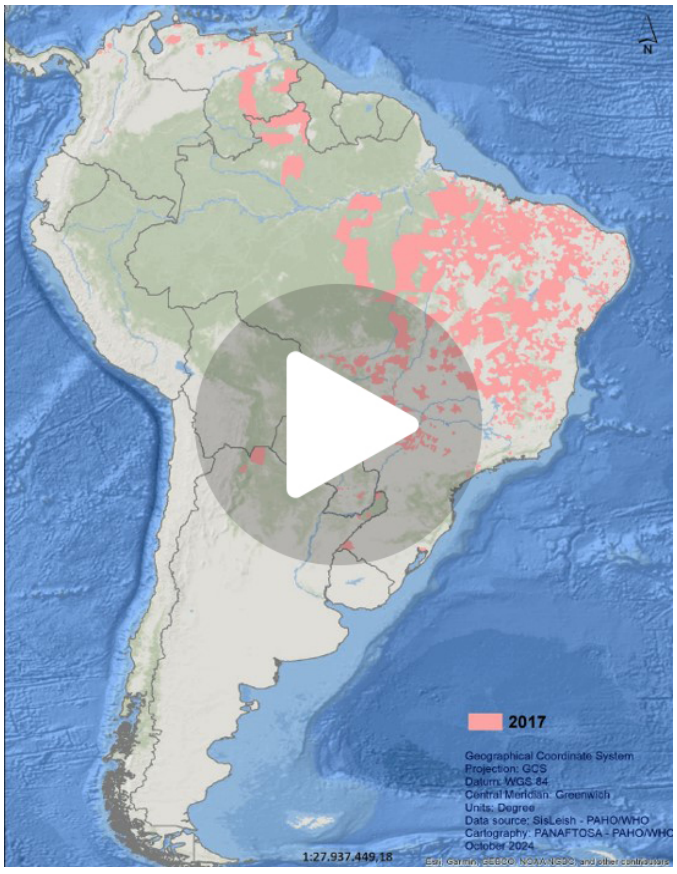


Note: Data reported by leishmaniasis programs and surveillance services in 11 endemic countries in the Region of the Americas. The graphed points correspond to counts of actual cases. The lines correspond to short-term trends, calculated by local regression.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

In 2023, 8 of the 13 countries with VL transmission reported cases to SisLeish. The cases are distributed among 56 units at the first administrative level and 631 units at the second level. A small year-to-year variation is observed between the number of administrative levels that reported cases; however, the geographic expansion of VL within and between countries continues. In 2023, this phenomenon was clearly observed on the borders between Argentina, the Plurinational State of Bolivia, Brazil, and Paraguay, with 93%, 50%, 3%, and 32% of VL cases, respectively.

Figures 11 and 12 map the occurrence of cases from 2017 to 2023, with 692 municipalities that had reported no cases since SisLeish was established in 2012. Figure 13 shows the border municipalities of Southern Cone countries with reported cases in 2023.

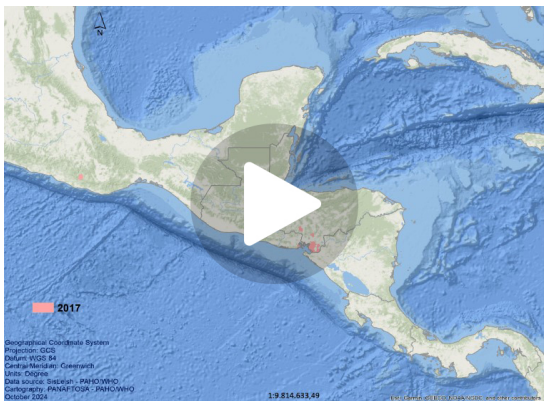
Figure 11. Historical series of reported visceral leishmaniasis cases at the second national administrative level, South America, 2017–2023



[If you cannot visualize the figure, click here]

Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Figure 12. Historical series of reported visceral leishmaniasis cases at the second national administrative level, Central America and Mexico, 2017–2023



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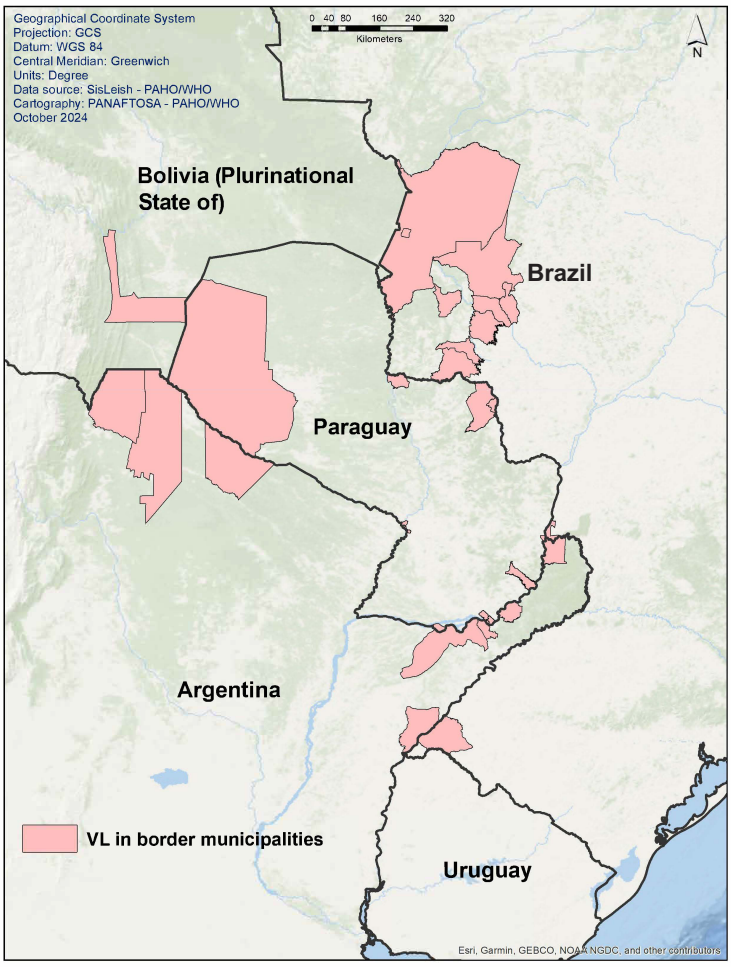
Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

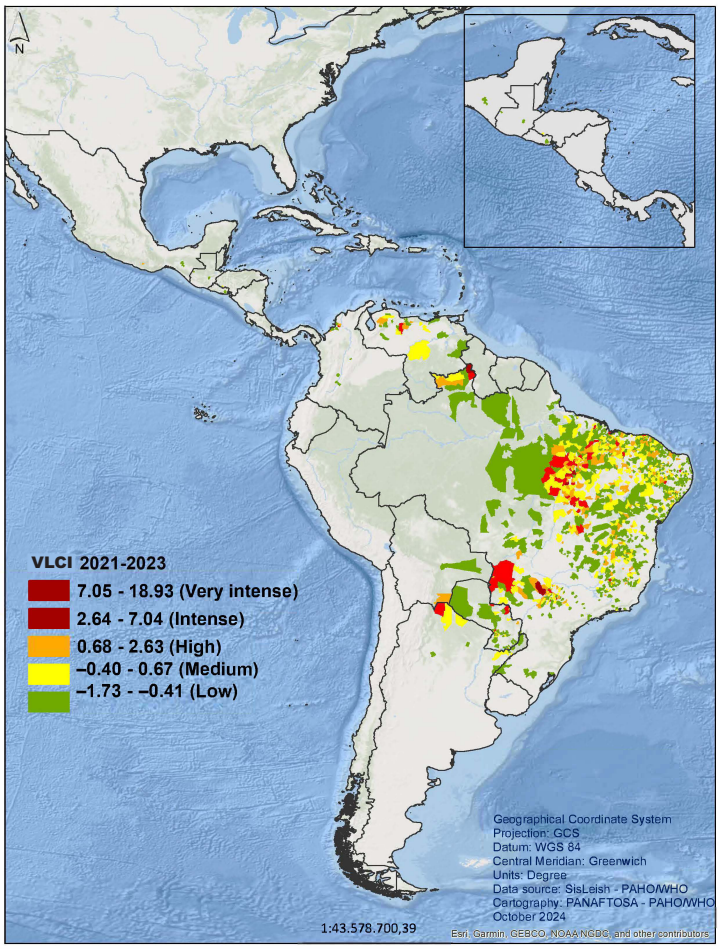
Figure 13. Cases of visceral leishmaniasis in municipalities on international borders in Southern Cone countries, 2023



VL: visceral leishmaniasis
Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Figure 14 shows VL risk stratification in the Region of the Americas at the second subnational administrative level, according to the composite indicator for the 2021–2023 triennium.

Figure 14. Stratification of visceral leishmaniasis risk at the second subnational administrative level according to the visceral leishmaniasis composite indicator, Region of the Americas, 2021–2023



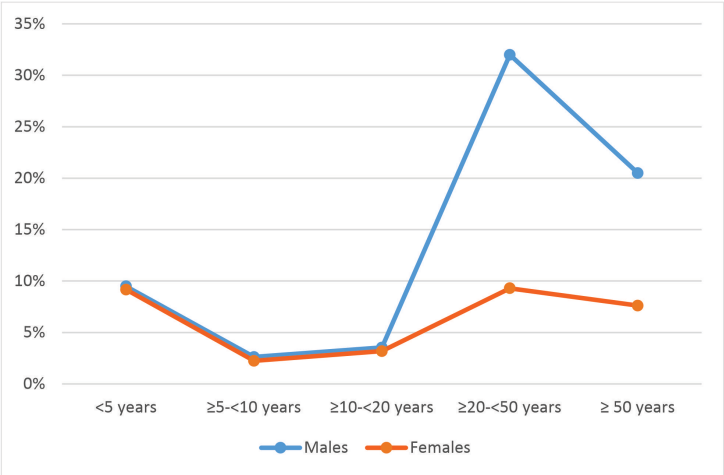
VLCI: visceral leishmaniasis composite indicator (average number of cases and case incidence per 100 000 population, 2019–2021).
Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

During this period, VL cases were reported in 1233 municipalities, with the following results for the municipalities with the highest risk of transmission: 7 units at the second level have very intense transmission (all in Brazil); 51 have intense transmission (1 in Argentina, 1 in Bolivia [Plurinational State of], 1 in Paraguay, and the rest in Brazil); 173 have high transmission (8 in Paraguay, 2 in Venezuela [Bolivarian Republic of], and the rest in Brazil).

Despite the reduction in the number of cases, there was a 20% increase in the number of municipalities with very intense, intense, and moderate transmission, driven by a 27% increase in the number of these municipalities in Brazil.

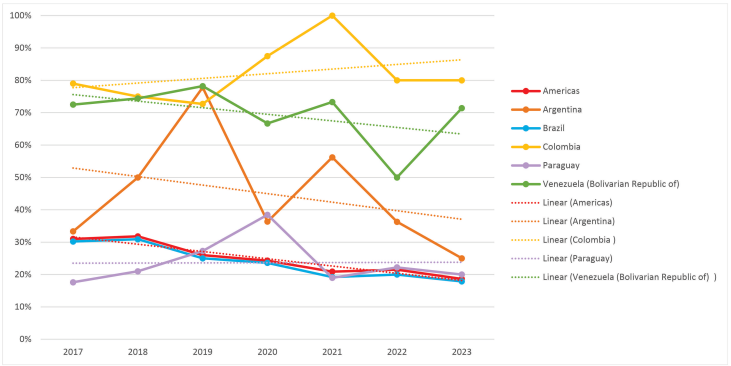
Sex was included as a variable in 100% of reported cases and age group was included in 99.7% of cases. The most affected group continues to be men between 20 and 50 years of age, which is reflected in the pattern observed in Brazil (Figure 15). However, among the countries with stable transmission from 2017 to 2023, there was an upward trend in cases among children under 5 years of age in Colombia and Paraguay, although the proportion fluctuated (Figure 16).

Figure 15. Proportion of visceral leishmaniasis cases by age group and sex, Region of the Americas, 2023



Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Figure 16. Proportion of visceral leishmaniasis cases in children under 5 years of age, Region of the Americas, 2023

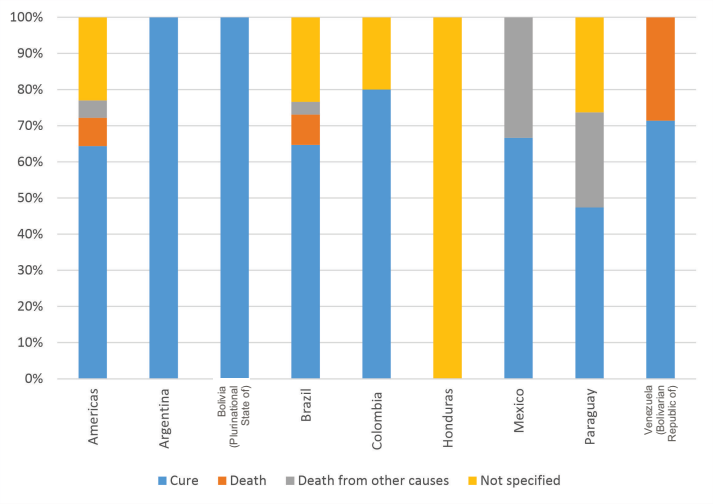


VL: visceral leishmaniasis
Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Brazil has the highest burden in the Region. However, it is important to note that its proportion has been decreasing over the years, while Argentina, Paraguay, and the Bolivarian Republic of Venezuela show an upward trend in the number of cases. In 2023, Brazil reported 91% of VL cases, the lowest proportion recorded in the 23 years studied, while Paraguay reported 6%, the highest proportion reported by any other country in the Region.

In total, 99.9% of reported VL cases included diagnostic confirmation criteria: 87% (1403 cases) were diagnosed by laboratory tests and 12% (199 cases) by clinical or epidemiological criteria, thus maintaining the general pattern observed in previous years. Of the patients with follow-up information available, 64.4% progressed to cure, 7.8% to death due to VL, and 4.8% to death due to other causes (Figure 17). Since 2017 there has been a reduction in the proportion of cured patients and an increase in the proportion of cases with no information on clinical outcome (23% in 2023, the second highest figure in the period studied; the highest proportion was 27% in 2020, during the COVID-19 pandemic).

Figure 17. Proportion of visceral leishmaniasis cases by clinical outcome, Region of the Americas, 2023

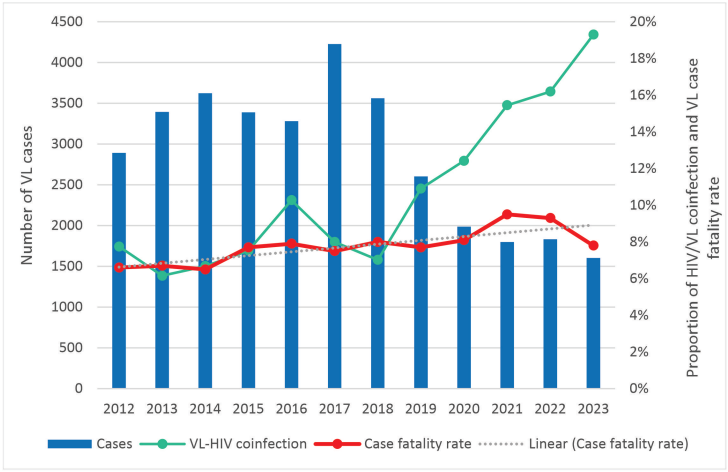


Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Despite a significant reduction in the number of VL cases in the Region, there is an upward trend in VL/HIV coinfection, with the highest proportion in 2023 (19%), due to cases reported in Argentina (7%), Brazil (20%), Paraguay (18%), and the Bolivarian Republic of Venezuela (14%). A small reduction in the VL case fatality rate was observed in 2022 and 2023 but the upward trend continues (Figure 18).

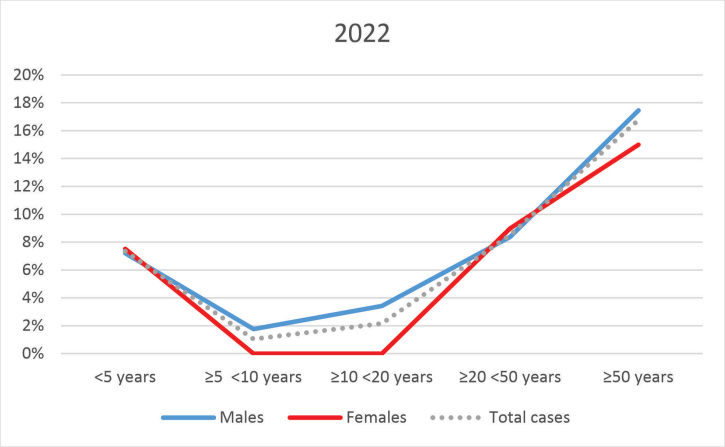
For both sexes, the age group with the highest case fatality rate continues to be 50 years and older (15% men; 16% women), followed by those 20–50 years, 10–20 years, and under 5 years, with similar case fatality rates in all groups (5%). However, when disaggregated by sex, a comparison of data for 2022 and 2023 shows that the rate for women is four times higher than for men in the 10–20 years age group and twice as high for girls under 5 than for boys (Figure 19).

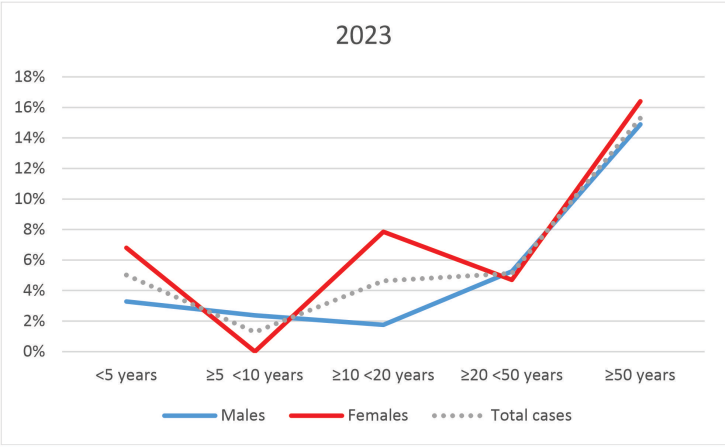
Figure 18. Proportion of visceral leishmaniasis and human immunodeficiency virus coinfection and visceral leishmaniasis case fatality rate, Region of the Americas, 2012–2023



VL: visceral leishmaniasis; HIV: human immunodeficiency virus
Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Figure 19. Case fatality rate for visceral leishmaniasis by age group and sex, Region of the Americas, 2022 and 2023





Note: Data reported by countries' leishmaniasis programs and surveillance services.
Source: Pan American Health Organization. Leishmaniasis Regional Information System (SisLeish). Washington, D.C.: PAHO; 2024 [cited 8 October 2024]. Limited access.

Final considerations

An analysis of cases in the Americas shows that the trend in cutaneous and visceral leishmaniasis has progressively declined in recent years. However, when the data are analyzed separately by country, cases of visceral leishmaniasis are trending upward in Argentina and Paraguay, as are cases of cutaneous leishmaniasis in Argentina, Costa Rica, Ecuador, Mexico, and Suriname.

The profile of CL cases follows the same pattern as in previous years, affecting mostly working-age adult men. At the same time, there has been a 39% reduction in the proportion of CL cases in children under 10 years of age compared to 2016. Countries are taking action to achieve the target of a 50% reduction in CL cases in this age group by 2030, in accordance with the *Plan of action to strengthen surveillance and control of leishmaniasis in the Americas 2023–2030 (3)*.

The mucosal form of leishmaniasis remains a persistent challenge in the Region: 2023 saw the highest proportion of reported cases since 2012. The countries with the highest number or proportion of cases are Paraguay, the Plurinational State of Bolivia, and Peru. It is necessary for the ministries of health to step up efforts aimed at early diagnosis, adequate treatment, and follow-up in referral services, since this clinical form of the disease requires strict monitoring due to lower treatment response and the possibility of evolving to severe forms with deformities or mutilations.

Over the years, the quality of the data reported in the PAHO Leishmaniasis Regional Information System has improved. However, there is still a need to work intensively with countries to improve the completeness of care-related data, such as data on treatments and clinical outcomes. This would allow monitoring and evaluation of the targets and indicators of the PAHO *Plan of action to strengthen surveillance and control of leishmaniasis in the Americas 2023–2030* and the WHO *Roadmap for neglected tropical diseases 2021–2030 (2, 3)*.

Despite the significant reduction in VL cases in the Region, the disease has spread considerably within countries and in international border areas in the Southern Cone countries, where the main transmission pattern is urban, and dogs are the main reservoirs. This facilitates the movement of infected animals to areas with a confirmed presence of the vectors that transmit *Leishmania infantum*, the parasite that causes the disease.

The group of patients most affected by VL in the Region continues to be men between 20 and 50 years of age, reflecting the profile of the disease Brazil, which accounted for 91% of cases in 2023. However, children under 5 years of age are the most affected group in Colombia (80%) and the Bolivarian Republic of Venezuela (71%).

Since 2018, VL/HIV coinfection continues on an upward curve, with the highest proportion in 2023 (19% of cases). Early diagnosis of HIV infection in patients with VL is important, as this group of patients requires specialized care and permanent follow-up with possible prophylactic treatment to avoid relapses of VL and further complications.

Another target of the *Plan of action to strengthen surveillance and control of leishmaniasis in the Americas 2023–2030 (3)* is to reduce the primary VL case fatality rate in the Region by 50% by 2030. This will require efforts by governments to make decentralized rapid testing available for diagnosis in primary care settings, and to include liposomal amphotericin B as the first-line treatment for pediatric and adult patients with this disease, following a strong GRADE (Grading of Recommendations Assessment, Development and Evaluation) recommendation *(4)*.

It is hoped that decision-makers will work jointly to make progress on the commitments made at the World Health Assembly and the PAHO Directing Council to improve the quality of care and the lives of those affected by this disease.

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Acknowledgments

PAHO thanks the professionals of the national leishmaniasis programs in disease-endemic countries, as well as those working in epidemiological, entomological, and laboratory surveillance programs, care services, and zoonotic disease services where leishmaniasis is endemic, and who participate directly or indirectly in strengthening surveillance, prevention, and control so that people with this disease in the Region of the Americas have access to better care.

Citation: Pan American Health Organization. Leishmaniasis: Epidemiological report on the Region of the Americas. No. 13, December 2024. Washington, D.C.: PAHO; 2024. Available from: https://iris.paho.org/handle/10665.2/51742.

PAHO/CDE/AFT/24-0021

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